less by ignorance than by anxiety. Dr Neha Aggarwal, MD Psychiatry at Fortis Hospital Manesar and Adayu Mindfulness, Gurugram, says health awareness is good: you look up symptoms, consider common causes and, if needed, consult a doctor. Cyberchondria, by contrast, she says, is "cyclical and distress-driven—hours of searching, quickly assuming the worst, seeking reassurance that never lasts and returning to the same fears. The line is crossed when searching for information no longer leads to clarity or action, but to escalating anxiety and repeated checking that begins to affect mental well-being." Most vulnerable are those who already have high health anxiety, a tendency to worry, perfectionistic traits or low tolerance for uncertainty. For them, she notes, "maybe it is nothing" is not enough. What she proposes is asking oneself a simple question: "Does your search end with a plan, or does it reopen anxiety?" If the pattern feels compulsive and difficult to stop, she says, professional support may be needed.

For many patients, the solution is not another test but addressing the anxiety underneath it. Experts say treatment often involves helping people step back from compulsive symptom-checking and the need for constant reassurance. "The goal is to build tolerance for uncertainty rather than eliminate every doubt," says Dr Upasana Chaddha, head psychologist, Mindscape Centre for Counseling, Delhi. Therapy is the mainstay, while yoga, mindfulness and stress-management practices further help reduce the anxiety that keeps the cycle going. For severe anxiety, psychiatrists may recommend sleep or anxiety medication.

MAKING CYBERCHONDRIA WORK

Cyberchondria is also not entirely negative, say doctors. Rather than mock or dismiss it, it must be redirected. Health anxiety should become an opportunity for preventive care, counselling and timely diagnosis. "These health anxieties create an important opportunity," says Dr Siddharth Shankar Sood. "When a worried individual visits the clinic, it allows me to counsel them, rule out serious disease, and initiate age-appropriate cancer screening based on their risk profile. If nothing serious is found, reassurance itself is therapeutic." The same goes for online advice—it is not automatically harmful. "Basic awareness around posture correction, stretching, and movement can be beneficial when applied correctly," notes Dr Jha; the real problem is the lack of personalisation and clinical evaluation. Greater vigilance can also save lives. According to Dr Sharma, it means some patients seek help earlier in genuine emergencies such as a stroke, where every minute matters.

Not all in the medical fraternity or the drugs industry blame technology either. Saransh Chaudhary, CEO, Venus Remedies, says the challenge is not technology itself since it can improve access, awareness and clinical decision-making when used responsibly. "The real question is whether its use is underpinned by transparent, traceable and regulated frameworks," he says. "With AI penetrating healthcare, there is a growing risk of consumers relying on partial or unverified information. This can have public health consequences, especially in an era of antimicrobial resistance driven by misuse and overuse of antibiotics."

Aditya Kandoi, founder & CEO, Redcliffe Labs, also lauds the roles of AI tools that encourage preventive screening or guide users toward the right tests. But his prescription, too, comes accompanied by a caveat: "Healthcare decisions require context, interpretation and medical judgement." AI, in his view, should support decisions, not become the diagnosis itself.

And under no circumstances should such a diagnosis progress to self-medication. According to Rohan Sanghvi, director of pharma distribution firm Shantilal Brothers, there is "rampant misuse of topical steroids, antifungals, painkillers and antibiotic combinations. Easy access to painkillers can mask underlying disease, while repeated or unsupervised use of common medicines can create its own health risks". He advocates tighter oversight of over-the-counter sales, especially antibiotics, steroids and certain anti-inflammatory drugs. He also believes that the neighbourhood pharmacy is crucial in breaking the cycle of self-diagnosis and self-medication. "Since pharmacists," says Sanghvi, "often see patients before doctors do, with the right training and protocols, they can identify red flags such as recurrent symptoms, unclear diagnoses or requests for high-risk drugs and redirect patients to doctors when needed. Their role can move from passive dispensing to active triaging, counselling and patient education."

Cyberchondria thrives because doctor appointments are difficult to get, and access to quality healthcare remains uneven, leaving patients with no recourse but to turn to the internet. They do get quick, easy answers, but ones that only stoke fears, not reassure an individual. If cyberchondria must be cured, credible medical advice has to be made even, and accessible. ■

COUNTERING THE MENACE

The action experts advise to regulate online medical advice

- **Regulators must strengthen oversight of prescription medicines and develop clearer guardrails for AI-generated health advice**
- **Doctors and pharmacists should help patients identify when basic awareness is tipping over into anxiety**
- **Tech platforms should emphasise the non-diagnostic nature of online advice and implement guidelines for health influencers**